

REGULATORY & COMPLIANCE

Subpoenas, Court Orders & Legal Requests: A Response Guide

What to do in the first hour, how to tell a subpoena from a court order, three ready letters, and how to keep your treating role separate from a forensic one.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Decision guide + letters

VERSION

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TIME TO ADAPT

45 minutes

WHAT IT IS

A calm, step-by-step guide for the moment a process server, a lawyer's letter or a licensing board notice arrives. It tells you what not to do, how to work out what the request is worth legally, and gives you the letters to reply.

USE IT WHEN

A subpoena, court order, attorney letter, police request, custody evaluator request or board inquiry has reached you, or you want a protocol ready before it does.

CUSTOMIZE FIRST

Your state's privilege and mental health records laws, your carrier's risk-line number, and your fee for legal work.

TWO RULES THAT PREVENT MOST HARM

Never ignore a subpoena, because missing a deadline can lead to court sanctions. And never hand over records the day it arrives, because a subpoena is not automatically a court order and the privilege belongs to your patient. Call your malpractice carrier's risk-management line the same day. Many carriers offer this free, and some provide an attorney.

Part A: The first hour

1. **Record the basics.** Date and time received, who served it, the court or agency, the case name and number, what was requested, and the response deadline. Use the log in Part D.
2. **Do not disclose anything yet.** Do not confirm that the person is your patient to anyone who does not already have a right to know. You may say you cannot confirm or deny anything without proper authority.
3. **Calendar the deadline and set reminders** well ahead of it.
4. **Secure the record.** Make a legal hold on everything about the patient, including notes, messages, portal messages, voicemail, calendar entries and any psychotherapy notes. Do not edit, add to or destroy anything.
5. **Call your carrier's risk line** and, where advised, an attorney who knows your state's mental health privilege law.
6. **Tell the patient,** unless counsel or the court advises otherwise. Ask what they want. The privilege belongs to them (see Part C letter).

7. **Respond in writing before the deadline** with the appropriate letter, and keep a record of exactly what you send.

Part B: What kind of request is it?

What you received	What it means and the usual next step
Court order signed by a judge	HIPAA permits disclosure of only what the order expressly authorizes. Read it carefully. If you think it is too broad, or the privilege has not been addressed, ask counsel about a motion to limit or quash. Log the disclosure.
Subpoena or discovery request, no court order (issued by a lawyer or clerk)	HIPAA allows a response only if you get written satisfactory assurance that the patient was notified and had time to object, or that a qualified protective order was obtained (or you seek one yourself), or you make reasonable efforts to notify the patient. You should also assert the psychotherapist-patient privilege on the patient's behalf unless the patient waives it or a court orders disclosure. Rules differ by state.
Patient's signed authorization	A valid HIPAA authorization lets you release what it describes, to the person it names. Check it: scope, recipient, dates, signature. Give only what it covers.
Attorney's letter asking for records	Not enough by itself. You need the patient's valid authorization, or a subpoena or court order handled as above.
Police or investigator	Ask for the legal basis in writing. A warrant or court order signed by a judge is different from a request or an administrative demand. Rules for law enforcement requests are specific and narrow. Involve counsel. In an emergency involving a serious threat, see the Suicide Risk and Safety Planning pack.
Substance use disorder treatment records (42 CFR Part 2)	A subpoena alone is not enough. A court order that meets Part 2's requirements is generally needed. Get advice.
Psychotherapy notes	HIPAA gives these extra protection, and many states also do. Do not release them without counsel's advice, even in response to a request that seems to cover "the whole record."
Custody evaluator, guardian ad litem or school	Treat as a third-party request. You need the patient's (or the legal decision-maker's) authorization or a court order. Watch for disputes between parents about who can consent.
Licensing board inquiry or complaint	Boards can generally get records for oversight without the patient's consent, and HIPAA permits this. Take deadlines seriously and see the checklist in Part F.
Claim or subpoena in a lawsuit involving you	Notify your carrier immediately, before you respond or speak with the other side.

PRIVILEGE AND CONFIDENTIALITY ARE NOT THE SAME THING

Confidentiality is your duty to keep information private. Privilege is a legal rule that lets the patient block disclosure in a court proceeding. Federal courts recognize a psychotherapist-patient privilege (Jaffee v. Redmond, 1996), and every state has its own version with exceptions. HIPAA compliance is not enough by itself, since you must also honor state privilege law.

Part C: Letters

Letter 1: To the requesting attorney (subpoena without a court order)

Re: [Case name and number]. Subpoena received [date].

Dear [Attorney name], I received your subpoena for records concerning [patient identifier]. I am unable to produce records at this time. Federal and state law protect this information, and the privilege belongs to the patient. Please provide either (a) a court order signed by a judge, (b) a valid written authorization signed by the patient, or (c) written satisfactory assurance, as HIPAA requires, that the patient has been notified of the request and has had an opportunity to object, or that a qualified protective order has been obtained. If none of these can be provided, I will object to the subpoena and ask the court to decide.

Nothing in this letter waives any privilege or objection. Please direct all further communication to [counsel or me, at ____]. Sincerely, [Name and credentials]

Letter 2: To the patient

Dear [Patient first name], I received a legal request on [date] from [name] in connection with [case description, if known]. It asks for [describe]. I cannot release your records in response to this request without your authorization or a court order, and I am required to protect the confidentiality of our work together. I wanted you to know right away, so that you can talk with your own attorney about whether you want to object or agree. Please contact me by [date] so we can discuss it. The deadline for a response is [date]. I will not release anything without a valid legal basis, and I will tell you what I release. Sincerely, [Name]

Letter 3: Cover letter when producing records under an authorization or court order

Re: [Case name and number]. Dear [Recipient], Enclosed are records concerning [patient identifier] released under [the attached signed authorization dated ____ / the court order dated ____]. This response includes only [describe scope] and does not include [material excluded, such as psychotherapy notes]. The enclosed records are confidential. Redisclosure is limited by law. I am available to answer questions regarding the scope of this response through [contact]. Sincerely, [Name]

KEEP A RECORD OF WHAT YOU SENT

Keep a copy of exactly what you disclosed and when, with a copy of the order or authorization. Disclosures for judicial proceedings must be listed in an accounting of disclosures if a patient asks for one.

Part D: Legal request intake log

Date and time received	Received from (name and role)	Delivery method
Court or agency, case name and number		Response deadline
What is being requested		
Type (court order, subpoena, authorization, other) and who signed		Patient notified on
Carrier or attorney contacted (name, date)		Legal hold in place on
Response sent (what, when and how)		Accounting entry made on

Part E: Your role: treating clinician, not forensic evaluator

Courts and lawyers often ask a treating clinician for opinions that a forensic evaluator is trained and independent enough to give: about custody, fitness, disability, competency or risk. Mixing these roles can harm the treatment, and your testimony. Decide your approach before you are asked, and put it in writing.

	Treating role	Forensic evaluator role
Purpose	Help the patient.	Answer a legal question for the court or a party.
Relationship	Patient is your client and trusts you.	The court or the party is the client. The examinee knows the limits of confidentiality.
Information	Mainly the patient's own account.	Many sources, records and tests, with corroboration.
Opinions	About diagnosis, treatment and progress.	About the legal question, with methods to support it.
Risk	An opinion for custody or capacity can damage the therapeutic alliance and expose you to complaints.	Designed for the role and its safeguards.

Treating clinician role statement (sample). I am [Patient]'s treating [psychologist / psychiatrist]. My role is to provide treatment. I can provide records with proper authorization or a court order and can testify as a fact witness about what I observed and did in treatment. I cannot serve as an independent evaluator or give opinions about [custody, parenting fitness, disability, or other legal questions], because that would conflict with my role as treating clinician. If an evaluation is needed, I can refer to a qualified forensic evaluator. My fee for time spent on legal matters is [\$___] per hour, payable in advance for testimony and record preparation.

Part F: If a licensing board complaint arrives

☐ Note the deadline. Boards are strict. Ask in writing for an extension if you need it.

- ☐ Call your carrier at once. Many policies include license-defense coverage and require prompt notice.
- ☐ Get an attorney who handles licensing matters in your state.
- ☐ Secure the record and put a legal hold on it. Do not change anything.
- ☐ Do not contact the patient about the complaint unless counsel advises.
- ☐ Write your response with counsel: factual, respectful, focused on the facts and the standard of care. Do not attack the complainant.
- ☐ Look after yourself. A complaint is stressful, and peer support helps (see the Consultation Group Charter).

IN PSYALLIANCE

Keep the phone numbers for your carrier risk line and your attorney in your credentials panel. Use the Town Hall to learn how colleagues in your state handle privilege questions, and never post anything that could identify a patient or a case.

Sources and further reading

- HHS, HIPAA Privacy Rule, 45 CFR 164.512(e) (judicial and administrative proceedings), 164.512(f) (law enforcement), 164.508(a)(2) (psychotherapy notes), and 164.528 (accounting of disclosures) ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- Jaffee v. Redmond, 518 U.S. 1 (1996): federal psychotherapist-patient privilege.
- 42 CFR Part 2, Subpart E: court orders authorizing disclosure and use of substance use disorder records.
- APA Specialty Guidelines for Forensic Psychology (2013), on the differences between treating and forensic roles ([apa.org](https://www.apa.org)).
- APA Ethics Code, Standards 4.05 (disclosures), 3.05 (multiple relationships) and 9.04 (release of test data) ([apa.org/ethics/code](https://www.apa.org/ethics/code)).

ABOUT THIS DOCUMENT

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